# ANAESTHESIA RECIPE GUIDE FOR OBSTETRIC SURGERY

**It is the responsibility of the prescriber to ensure that there are no contraindications to the following:**

## Premed:

'Pre-Op' carbohydrate drinks, **except for diabetic parturients** (can be taken up to 2 hours prior to surgery):

- For morning theatre cases: 4 cartons at 22:00 on evening prior & 2 cartons at 07:00 on morning of surgery
- For afternoon theatre cases: 2 cartons at 11:30am on day of surgery

Omeprazole 40mg PO on evening prior and morning of day of surgery

Sips of water (no more than 30ml / hour) for the 2 hour period immediately prior to theatre

30ml 0.3M Sodium Citrate PO on arrival in theatre

## Anaesthetic:

### Regional Anaesthesia (RA):

1. Spinal anaesthesia* using 0.5% Hyperbaric bupivacaine + intrathecal opioid (preferably a long acting opioid e.g diamorphine)

   * 2.5ml Hyperbaric bupivacaine + 300mcg diamorphine is usually recommended for a routine Caesarean Section in a patient of average height

2. Epidural Top up Anaesthesia using 15 – 20ml 0.5% Levobupivacaine plus 100mcg fentanyl

   or 15 – 20ml 2% Lignocaine (+ 1:200,000 adrenaline) plus 100 mcg fentanyl

**or General Anaesthesia (GA)**

## Pre-incision:

Antibiotics: Cefuroxime 1.5G & Metronidazole 500mg (unless previous anaphylaxis to penicillin)

## Intra-op:

### For RA & GA cases :

Maintain normothermia using an in-line intravenous fluid warmer & ensuring the ambient theatre temperature is 23°C

Avoid excessive intravenous fluid administration

Following delivery administer : Paracetamol 1g iv (or 15mg/Kg <50Kg)

Ondansetron 4mg iv

Dexamethasone 0.1mg/Kg **(except for diabetics)**

Diclofenac 100mg PR at the end of surgery if NSAIDS not contraindicated e.g. patient intolerance, pre-eclampsia, HELLP or haemorrhage > 1 litre

### For Epidural top up cases only :

Diamorphine 3mg epidurally at end of surgery

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**For GA cases only :**

Maintain adequate muscle relaxation during surgery to optimise surgical access (use TOF monitoring)

Morphine up to 10mg iv in boluses as required

Bilateral TAP blocks (30ml 0.25% Levobupivacaine each side (or 20ml each side if body weight < 50Kg) to be administered either by the obstetrician during surgery or by the anaesthetist following surgery)

**Post-op:** ***For postop. analgesia advice call Obs. Anaesthetist (pager 551)***

**Regular** :

**For RA & GA cases:**

Paracetamol 1g (or 15mg/Kg if body weight < 50Kg) qds PO / iv

Ibuprofen 600mg qds PO

(Avoid NSAIDS in pre-eclampsia & HELLP Syndrome. Prescribe with caution following obstetric haemorrhage. Instead prescribe Dihydrocodeine 30mg qds PO (if breastfeeding) or Tramadol 100mg qds PO (if not breastfeeding))

Lactulose 15ml bd PO

Senna 15mg nocte PO

**Plus for GA cases only add:**

Oxygen (while on morphine PCA)

Ondansetron 4mg tds iv / im / PO

**PRN** :

**RA cases only:**

Oral Morphine Solution 25mg 2 hourly PRN (15mg 2 hourly PRN if opioid sensitive or body weight < 50Kg)

Ondansetron 4mg 8 hourly iv / im / PO

**GA cases only:**

Morphine PCA (1mg bolus, 5 minute lockout)

Cyclizine 50mg 8 hourly im or iv slowly

Naloxone 100mcg iv up to 4 times at 2 minute intervals

**Intravenous fluid :**

**For RA & GA cases:**

Hartmanns solution @ 83ml/hour until adequate oral fluid intake, but needs to continue if morphine PCA connected

***To be prescribed by the surgical team :*** Enoxaparin (if required)

**Analgesia TTO for discharge :**

Paracetamol 1g (or 15mg/Kg if body weight < 50Kg) qds PO

Ibuprofen 600mg qds PO *(If NSAIDS contraindicated substitute with: Dihydrocodeine 30mg qds PO (if breastfeeding) or Tramadol 100mg qds PO (if not breastfeeding)*